NCCP 00485 · VERSION 6

Your treatment, explained

Adjuvant or neoadjuvant high-risk node-negative or node-positive breast cancer · NCCP 00485 v6 · Document revised 4 November 2026

Your personal plan: this is the standard regimen schedule. Your team confirms your prescribed doses, cycles, treatment changes and appointments in your written plan.

Dose-dense AC → weekly paclitaxel

14-day cycle

Source reconciled; independent clinical, pharmacy and patient/PPI review pending

ACT ON SYMPTOMS, NOT THE CALENDAR

When to get help

Emergency: call 112 / 999

Severe breathing difficulty, collapse, seizure or severe chest pain: call emergency services. Do not wait for an online answer.

Contact your oncology service immediately

Temperature 37.5°C or higher, 36°C or lower, shivering or suddenly feeling unwell — even with a normal temperature. Report new breathlessness, jaundice, marked bleeding, severe diarrhoea, inability to drink, chest symptoms or a severe rash promptly.

Follow the urgent contact instructions from your treating hospital. If you cannot reach the team, seek urgent medical assessment. Outside clinic hours, use your instructed out-of-hours pathway or the emergency department.

This page does not diagnose symptoms or decide whether it is safe to stay at home. You do not need to know which medicine caused a symptom before calling.

Open your CUH contact / treatment card

Sources: nccp · hse

YOUR COMPLETE REFERENCE

Read what you need, when you need it.

Start with the overview, explore the cycle, or go straight to a question. Urgent advice is always available.

- How each drug works
- Treatment and review
- Your 14-day timeline
- Baseline and cycle-specific checks
- Whole treatment, scans and checks
- Tablets, visits and practical planning
- Why this treatment?
- Checks across treatment
- The wider treatment plan
- Supportive medicines
- Questions to ask
- Sources

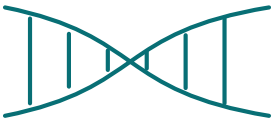
UNDERSTAND YOUR TREATMENT

Your medicines and what they do

Reduce recurrence risk as part of the agreed early-breast treatment plan.

ANTHRACYCLINE CHEMOTHERAPY

Doxorubicin



Interferes with DNA handling

Doxorubicin interferes with enzymes and DNA processes needed for cell division. It can also affect heart muscle, so the team considers previous exposure and cardiac risk.

DNA handling disrupted → Cell damage → Division reduced

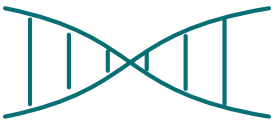
What to watch for

Report pain or burning at the infusion site immediately. New breathlessness or chest symptoms need prompt review. Red urine for a short time can be a drug-colour effect; pain, clots or persistent bleeding still need advice.

Sources: nccp

ALKYLATING CHEMOTHERAPY

Cyclophosphamide



Damages DNA needed for growth

Cyclophosphamide is activated in the body and damages DNA. This helps stop dividing cancer cells but can also affect blood-cell production and reproductive tissues.

Drug activated → DNA damage → Cell division affected

What to watch for

Use the prescribed anti-sickness and hydration plan. Report fever, painful urination or blood in the urine.

Sources: nccp

TAXANE CHEMOTHERAPY

Paclitaxel

Taxol (conventional paclitaxel)



Locks the cell division machinery

Cells use tiny structures called microtubules to separate their chromosomes when dividing. Paclitaxel stabilises these structures so they cannot rearrange normally, interrupting cell division. It can also affect nerves and other healthy tissues.

Microtubules form → Structures remain locked → Division is interrupted

What to watch for

Report tingling or numbness before the next weekly dose, especially if it affects walking or hand use. Tell the nurse immediately about flushing, chest or throat tightness, itching, dizziness or breathing changes during infusion.

Sources: nccp

The drawings are simplified explanations of drug action, not anatomical pictures or a prediction of response.

Explore your existing anatomy and side-effect map

THE BIGGER TREATMENT PLAN

Treatment and review

4 CYCLES, EVERY 14 DAYS

AC chemotherapy

4 cycles, every 14 days

Growth-factor support is required for each dose-dense AC cycle. The anti-sickness prescription belongs to this phase.

12 WEEKLY DOSES

Weekly paclitaxel



Premedicines and anti-sickness support change. Bloods and numbness are checked before each weekly dose.

Your team confirms the active medicines, treatment days, dose changes and next review.

Sources: nccp

BASELINE → TREATMENT → FOLLOW-UP

When checks happen

Relative milestones, not equal time intervals. Your team books tests and reviews results; this map does not confirm completion.

Check	Before starting	First treatment	During treatment	Change / completion	Follow-up
Blood / organ function	FBC, renal, liver baseline	Review results before AC	FBC, kidney and liver tests before each cycle; team confirms checks before weekly paclitaxel doses	Confirm recovery before restart / transition	Investigate persistent problems
Heart / nerves	ECHO/MUGA if >65 or indicated; doxorubicin LVEF >50%	Review symptoms / anthracycline exposure	Cardiac review as indicated; taxane neuropathy	Review cumulative dose and symptoms	Report new cardiac symptoms
Support changes	Agree growth-factor product / days	G-CSF required during dose-dense AC	Switch to weekly-paclitaxel premedicines	Do not carry AC support across automatically	Review continuing support needs

The team arranges and reviews tests. This guide shows expected timing; it does not record completed appointments. NCCP 00485 v6 · official regimen · ESMO 2024 early breast cancer guideline · treatment and follow-up · NICE NG101 · bone health and follow-up

THE WHOLE TREATMENT JOURNEY

Dose-dense AC → weekly paclitaxel: treatment and checks

Adjuvant or neoadjuvant high-risk node-negative or node-positive breast cancer

Four AC treatments (doxorubicin 60 mg/m² and cyclophosphamide 600 mg/m²) 14 days apart, followed 14 days after the last AC by 12 weekly paclitaxel 80 mg/m² doses, subject to recovery. The planned sequence spans about 20 weeks.

Read from left to right. Columns show stages, not equal lengths of time. This is a general plan: no dates or personal information are needed. Your team confirms which steps apply and arranges appointments.

On a small screen, swipe the map sideways. The checks below give the same timing in words.

Treatment, tests and reviews across the course

Care / check	Before starting	First treatment	During treatment	Change / completion	Follow-up
Treatment plan NCCP 00485	Eligibility and cardiac risk	AC q14 days ×4	Paclitaxel weekly ×12	Agree local/endocrine treatment	Early-breast follow-up
Blood / organ function NCCP 00485	FBC, renal, liver baseline	Review results before AC	FBC, kidney and liver tests before each cycle; team confirms checks before weekly paclitaxel doses	Confirm recovery before restart / transition	Investigate persistent problems
Heart / nerves NCCP 00485	ECHO/MUGA if >65 or indicated; doxorubicin LVEF >50%	Review symptoms / anthracycline exposure	Cardiac review as indicated; taxane neuropathy	Review cumulative dose and symptoms	Report new cardiac symptoms
Support changes NCCP 00485	Agree growth-factor product / days	G-CSF required during dose-dense AC	Switch to weekly-paclitaxel premedicines	Do not carry AC support across automatically	Review continuing support needs
Cancer review EARLY	Confirm setting	Agree review plan	Follow the early-breast plan	Next treatment / completion	Agreed surveillance

What each check is for

1

Before treatment

Before the first dose

Adjuvant or neoadjuvant high-risk node-negative or node-positive breast cancer. FBC, kidney and liver tests at baseline and before each cycle, with the unit confirming weekly paclitaxel test requirements. Review current illness and past dose delays separately.

Ask / act: Confirm eligibility, test results and medicine list.

Sources: NCCP 00485

2

Other checks

At the specified review points

Review lifetime anthracycline exposure and cardiac risk. Echo/MUGA is indicated above age 65 or when clinically indicated; the source requires LVEF above 50% for doxorubicin. Reassess cardiac symptoms and neuropathy as indicated; no trastuzumab is included.

Ask / act: Confirm who arranges and reviews each check.

Sources: NCCP 00485

3

Support and practical plan

At each treatment or phase change

G-CSF is required for every dose-dense AC cycle. Prescribe product, dose and timing separately under the approved local plan. First paclitaxel dose: dexamethasone 8 mg IV, chlorphenamine 10 mg IV and famotidine 20 mg IV, each 30 minutes before treatment (NCCP suggested table).

Ask / act: Choose the matching support plan; confirm active medicines.

Sources: NCCP 00485

4

Response and follow-up

Agreed review points

After early-breast treatment, breast imaging and clinical review follow the agreed plan. Routine CT/PET scans or blood tumour markers are not used to search for distant recurrence in someone without symptoms or clinical concerns. Keep the agreed follow-up and report persistent or new symptoms between visits.

Ask / act: Review symptoms, results and patient priorities.

Sources: EARLY

At the next review, ask

- Which treatment phase am I in, and what happens next?
- Which test is expected next, who arranges it, and who explains the result?
- Who should I contact if an expected appointment has not arrived?

This map does not show whether a test has been ordered, booked or completed. Do not wait for a scheduled test if you develop new symptoms.

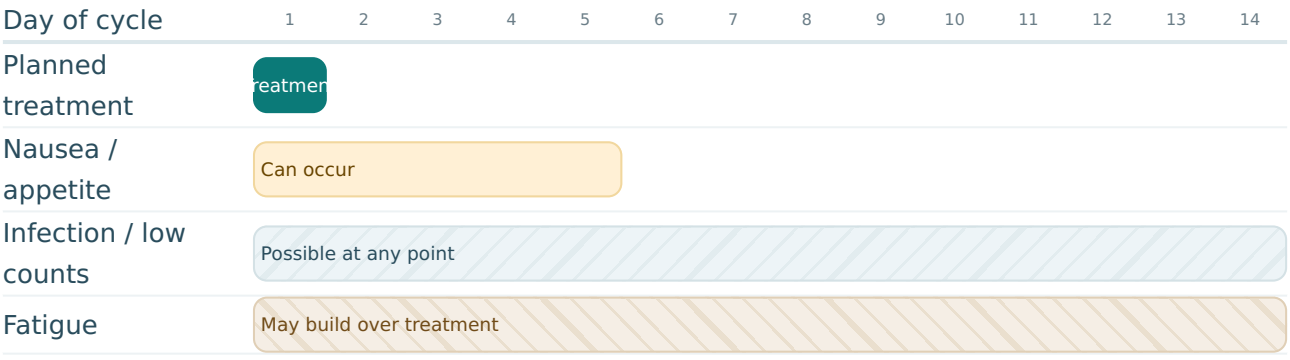
Sources checked 2026-10-25 · Source reconciled; independent clinical, pharmacy and patient/PPI review pending · SACTCheck v0.84.19

Phase 1 · AC every 14 days

Day 1 → Day 14: a general pattern

This is a visual orientation, not a prediction of your personal cycle. It shows when some effects often emerge and which effects are **not** tied to a particular day.

Four AC treatments 14 days apart, followed 14 days after the last AC by 12 weekly paclitaxel doses, subject to recovery and review.



Timing varies: Symptoms can occur on any day, including days without treatment. The bands are orientation, not predicted symptom-free periods.

Treatment-day changes: A delayed, omitted or changed dose needs an updated plan from the treating team. Do not move doses or restart on your own.

> **Why these bands stay broad:** Planned treatment days are distinct from broad possible symptom patterns. Symptoms can occur outside these bands. Band length does not show likelihood or severity, and the chart cannot tell you whether it is safe to receive treatment. NCCP 00485 v6 · official regimen ↗

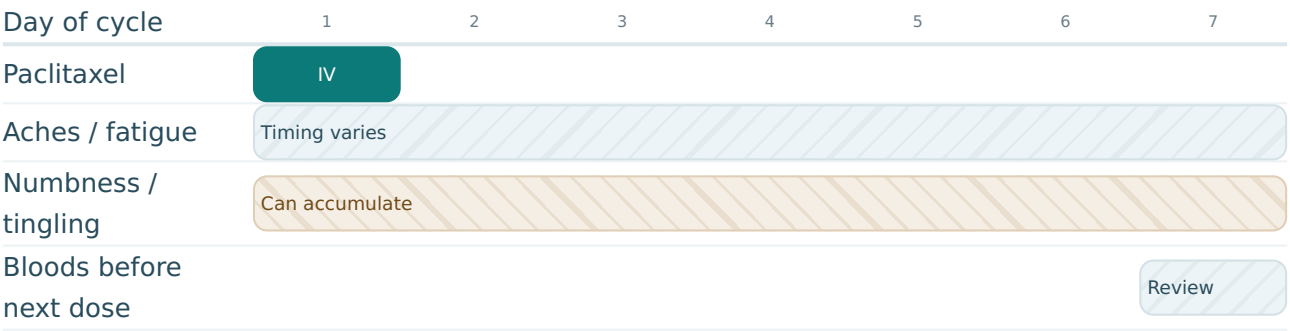
> **There are no guaranteed “safe days”.** Fever, shivers, sudden illness, new breathing problems or severe symptoms need prompt advice on any day, including a treatment break.

Phase 2 · weekly paclitaxel

Day 1 → Day 7: a general pattern

This is a visual orientation, not a prediction of your personal cycle. It shows when some effects often emerge and which effects are **not** tied to a particular day.

Paclitaxel is repeated weekly for 12 doses.



Timing varies: Symptoms can occur on any day, including days without treatment. The bands are orientation, not predicted symptom-free periods.

Treatment-day changes: A delayed, omitted or changed dose needs an updated plan from the treating team. Do not move doses or restart on your own.

> **Why these bands stay broad:** Planned treatment days are distinct from broad possible symptom patterns. Symptoms can occur outside these bands. Band length does not show likelihood or severity, and the chart cannot tell you whether it is safe to receive treatment. NCCP 00485 v6 · official regimen ↗

> **There are no guaranteed “safe days”.** Fever, shivers, sudden illness, new breathing problems or severe symptoms need prompt advice on any day, including a treatment break.

Read the whole cycle as text

Day 1–14: Reference cycle

Four AC treatments (doxorubicin 60 mg/m² and cyclophosphamide 600 mg/m²) 14 days apart, followed 14 days after the last AC by 12 weekly paclitaxel 80 mg/m² doses, subject to recovery. The planned sequence spans about 20 weeks. Follow your written appointment and medicine plan. A calendar does not authorise a dose.

PRACTICAL TREATMENT GUIDE

Your treatment and access plan

Four AC treatments (doxorubicin 60 mg/m² and cyclophosphamide 600 mg/m²) 14 days apart, followed 14 days after the last AC by 12 weekly paclitaxel 80 mg/m² doses, subject to recovery. The planned sequence spans about 20 weeks.



STEP 1

Before treatment

FBC, kidney and liver tests at baseline and before each cycle, with the unit confirming weekly paclitaxel test requirements. Review current illness and past dose delays separately.



STEP 2

Plan venous access

A cannula, PICC or implanted port may be used. A port is not automatically required; insertion and removal are separate procedures with individual instructions.



STEP 3

On the treatment day

Confirm the medicines due that day, any premedicines and the reviewed blood results. Tell staff immediately about pain, burning, rash or breathlessness.



STEP 4

After the visit

No routine take-home infusion pump is required for this regimen. Keep line-care, urgent-contact and next-review instructions.

Visit length

AC and weekly paclitaxel have different visit and supportive-medicine plans. The unit confirms the complete appointment duration.

Line care

A PICC or port has its own care plan. Report fever, redness, swelling, pain, leakage or line damage; do not remove or adjust it yourself.

Treatment changes

A delayed, omitted or changed dose needs an updated plan from the treating team. Do not move doses or restart on your own.

Support for this visit

G-CSF is required for every dose-dense AC cycle. Prescribe product, dose and timing separately under the approved local plan. First paclitaxel dose: dexamethasone 8 mg IV, chlorphenamine 10 mg IV and famotidine 20 mg IV, each 30 minutes before treatment (NCCP suggested table).

Open practical treatment guide · **Printable practical guide**

Sources: NCCP 00485 v6 · official regimen · eviQ · implanted port patient information · eviQ · PICC patient information. Source reconciled; independent clinical, pharmacy and patient/PPI review pending.

MAKE THE PLAN PRACTICAL

Support around the medicines

1 Blood counts and infection

Fever, shivers or sudden illness needs prompt advice on any day.

2 Nerve symptoms

Report numbness, tingling or weakness before the next dose.

3 Support medicines

Follow the prescription for the active medicine and treatment day.

4 **Review the plan**

Discuss symptoms, recovery and the next treatment decision with the team.

Sources: nccp · antiemetic

TREATMENT REFERENCE · 01

Why this treatment?

This anthracycline–taxane sequence is an option for selected high-risk early breast cancer. Dose-dense AC needs growth-factor support; the later weekly paclitaxel phase has different premedicines and toxicity priorities.

Your treatment setting

Adjuvant or neoadjuvant high-risk node-negative or node-positive breast cancer

Sources: nccp

Your exact schedule

Four AC treatments (doxorubicin 60 mg/m² and cyclophosphamide 600 mg/m²) 14 days apart, followed 14 days after the last AC by 12 weekly paclitaxel 80 mg/m² doses, subject to recovery. The planned sequence spans about 20 weeks.

Sources: nccp

Alternatives

Discuss biomarker-directed, endocrine, chemotherapy and supportive-care options appropriate to this setting, including the option not to proceed.

Sources: nccp

Checks across treatment

Your team arranges tests and reviews the results before treatment decisions.

Blood and organ checks

FBC, kidney and liver tests at baseline and before each cycle, with the unit confirming weekly paclitaxel test requirements. Review current illness and past dose delays separately.

Sources: nccp

Other treatment-specific checks

Review lifetime anthracycline exposure and cardiac risk. Echo/MUGA is indicated above age 65 or when clinically indicated; the source requires LVEF above 50% for doxorubicin. Reassess cardiac symptoms and neuropathy as indicated; no trastuzumab is included.

Sources: nccp

Cancer review and follow-up

After early-breast treatment, breast imaging and clinical review follow the agreed plan. Routine CT/PET scans or blood tumour markers are not used to search for distant recurrence in someone without symptoms or clinical concerns.

Sources: early

Bone health

Bone-health assessment follows endocrine therapy, ovarian suppression, menopause and other fracture risks. It has its own monitoring plan.

Sources: bone

TREATMENT REFERENCE · 03

The wider treatment plan

For neoadjuvant treatment, agree response review and surgery planning. After surgery, confirm radiotherapy and endocrine treatment where appropriate; their schedules and checks remain separate.

Treatment changes

A delayed, omitted or changed dose needs an updated plan from the treating team. Do not move doses or restart on your own.

Sources: nccp

Medicines review

Check new prescriptions, over-the-counter medicines and supplements with the oncology pharmacist before taking them.

Sources: nccp

After treatment

Keep the agreed follow-up and report persistent or new symptoms between visits.

Sources: nccp

Why this treatment may be chosen

This anthracycline–taxane sequence is an option for selected high-risk early breast cancer. Dose-dense AC needs growth-factor support; the later weekly paclitaxel phase has different premedicines and toxicity priorities.

Discuss how this evidence and the available alternatives apply to your own situation.

Sources: nccp

PREPARE FOR THE CONVERSATION

Questions worth asking

Why was this treatment chosen for me?

This anthracycline–taxane sequence is an option for selected high-risk early breast cancer. Dose-dense AC needs growth-factor support; the later weekly paclitaxel phase has different premedicines and toxicity priorities.

What happens if a dose is delayed or missed?

A delayed, omitted or changed dose needs an updated plan from the treating team. Do not move doses or restart on your own.

Who arranges the checks and explains the results?

Agree the due window, booking team, result reviewer and contact route if a test or appointment is missing. This page does not record completed checks.

Can I take other medicines or supplements?

Check new prescriptions, over-the-counter medicines and supplements with the oncology pharmacist before taking them.

> **Try explaining the plan back:** Which medicines and doses are prescribed, when are they due, which checks are needed, and what would make you call the team today?

SOURCE TRANSPARENCY

Where this information comes from

Regimen schedule: NCCP 00485 v6. Medicine explanations and safety information: sources below. The timeline is an educational illustration, not a measured risk curve or an individual prediction.

- NCCP 00485 v6 · official regimen
- ESMO 2024 early breast cancer guideline · treatment and follow-up
- ESC 2022 cardio-oncology guideline · cardiac surveillance and risk
- HSE · chemotherapy side effects and urgent advice
- NICE NG101 · bone health and follow-up
- NOGG · section 7, aromatase inhibitors and fracture risk
- eviQ · implanted port patient information
- eviQ · PICC patient information
- NCCP NCIS antiemetics v8 (2025), Table 1
- ASCO · biomarkers in metastatic breast cancer
- ESMO metastatic breast cancer guideline · 2026

SACTCheck · Developed by Dr Paul O’Brien · Sources checked 2026-10-25.
This information supports your treatment discussion. Your oncology team confirms your personal treatment and urgent-care plan.

Written source addresses

https://healthservice.hse.ie/documents/6500/485_v6_Dose_DenseAC-T.pdf

https://www.annalsofoncology.org/article/S0923-7534(23)05104-9/fulltext

https://www.nice.org.uk/guidance/ng101/chapter/Recommendations

https://www.eviq.org.au/p/507

https://www.eviq.org.au/p/543

https://academic.oup.com/eurheartj/article/43/41/4229/6673995

https://www2.hse.ie/conditions/chemotherapy/side-effects/

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https://healthservice.hse.ie/documents/7180/Supportive_Care_Antiemetics_for_inclusion_NCIS_Medical_Oncology.pdf

https://ascopubs.org/doi/10.1200/JCO.2015.61.1459

https://www.annalsofoncology.org/article/S0923-7534(26)00902-6/fulltext